



PREPARED FOR AVOCADO HEALTH · THURSDAY, SEPTEMBER 10, 2026

Avocado Verified Intelligence

Virginia's Innovative Maternal Care RFA is live with a deadline, an amount and a telephone number — the best-shaped opportunity on this file.

Daily digest. Ten dated opportunities, six in the pipeline, twenty-five published contacts.

DATED
OPPORTUNITIES

10

On the deadline calendar

EMERGING PIPELINE

6

Announced, no published date

PARTNER TARGETS

6

Award winners, Section 6

PUBLISHED CONTACTS

25

19 named individuals

NEXT DATE

Sep 14

4 days — VA Provider Productivity

#1

VA Innovative Maternal Care

\$11.6M available, awards to \$1M. Avocado cannot hold the award; the route is partner to a lead applicant — and there is real time to build one.

Pre-application conference Sep 16 · Closes Oct 6

#2

Comagine Health (ID)

Confirmed Idaho awardee on maternal and child health, with a named executive. Comagine improves the encounter; the gap is contact between encounters.

Call 800-949-7536 · Dana Hawes

#3

VA Live Well, Together

The only instrument that writes the technology partner into the eligible-applicant structure. No RFA yet — which is why you ask now.

Email Michael Parsons this week

How to read this edition. Section 1 is the action list and ignores rank. Sections 3 and 5 separate what has a published deadline from what does not; a forecast is never a deadline. Every contact in Section 8 is published in the clear — where a site obscures an address, this document says so and constructs nothing. **These are recommendations. Nothing has been sent and no one has been contacted.**

Prioritized to-do list

#1 Virginia Innovative Maternal Care — Fit 9/10 — Advance planning (26 days). Register for the **pre-application conference, September 16, 2:00 p.m. ET (Webex) — 6 days.** Call **Brandon Rivenbark, Rural Hospital Program Manager, VDH, (804) 835-2618, brandon.rivenbark@vdh.virginia.gov**. Avocado cannot apply directly; ask which lead applicants want a digital health partner — the RFA encourages collaborative applications under one lead. Send a short capability overview covering SMS-first delivery without an app. → Card, Section 4

#2 Comagine Health (ID) — Fit 9/10 — Advance planning. Call Comagine's Seattle office, 800-949-7536; ask for **Dana Hawes, Chief Growth Officer**. Ask whether the Idaho OB Readiness scope includes between-visit contact with pregnant and postpartum patients, and who selects delivery partners. No email is published for her. → Card, Section 4

#3 Virginia Live Well, Together — Consumer Technology — Fit 9/10 — Advance planning. Email **Michael Parsons, Michael.Parsons@healthyyouthva.org**. Ask when the Consumer Technology RFA publishes and whether a technology company can be named as partner at application rather than after award. Send a short capability overview. → Card, Section 4

#4 Missouri ToRCH local delivery partner round — Fit 8/10 — Advance planning. Email **DSS.RHTP@dss.mo.gov**. Ask when the regional coordinating network "Healthier Communities Together" opens its Fall 2026 application period for local delivery partners, and how vendors are selected. → Pipeline, Section 5

#5 New York RCHI awardee cohort — Fit 7/10 — Advance planning. NYSDOH publishes the awardee list with lead applicants, counties and award amounts. Use it to identify the highest-fit organizations. → Card, Section 6

#6 Virginia CareIQ Provider Productivity — Fit 7/10 — Short runway (4 days). Email **Ben Barber, ben@vhcf.org**. One question: is a caregiver-facing messaging layer that absorbs routine after-hours questions fundable under the aim of reducing inbox management, or does it belong in the Consumer Technology track? Four days is too short for a new partner bid; ask for the answer and the next round. → Card, Section 4

#7 Virginia Mobile & Hybrid Care — Fit 7/10 — Limited runway (8 days). Email **Brandon Davis, bdavis@nsvregion.org**. The RFA names "mobile health technology organizations" as eligible applicants; ask whether a text-based support layer attached to a mobile unit qualifies. → Card, Section 4

#8 Virginia CareIQ Remote Patient Monitoring — Fit 7/10 — Short runway (5 days). Email **Vicki Humphreys, vhumphreys@vhha.com**. Ask which eligibility statement controls for Budget Period 1: the RFA limits applicants to hospitals and health systems; the web page lists a wider set. The answer decides whether a route exists. → Card, Section 4

#9 **New Hampshire GO-NORTH Rural Primary Care Transformation — Fit 6/10 — Advance planning (35 days).** Call **Travis Boucher, Vice President, Rural Health Transformation, (603) 415-4258.** Applications roll after October 15. Ask which applicants are short on the technology-adoption focus area. His email is not published; the program address is GO-NORTH-FHC@healthynh.org. → Card, Section 4

#10 **Idaho Behavioral Health Prevention — Fit 6/10 — Deadline not published.** Email IdahoRHTP@dhw.idaho.gov; request the solicitation packet in readable form and the application deadline. The state lists no due date and the packet is unreadable; fit cannot be substantiated until it is read. → Worth Qualifying, Section 4

#11 **Idaho SUD Prevention — Fit 6/10 — Deadline not published.** Same request, same address. → Worth Qualifying, Section 4

These are recommendations. Nothing has been sent and no one has been contacted.

TODAY'S READ

This edition's read

2

The deadline calendar holds **ten opportunities**; the Emerging Pipeline **six more**. Section 6 carries **six award-winner or partner-target cards**, with Comagine Health promoted into the priority cards. Section 8 lists **twenty-five published contact rows**, nineteen of them named individuals.

THE HEADLINE

- **Virginia's Innovative Maternal Care RFA is live, with a deadline, an amount and a telephone number. It is the best-shaped opportunity on this file.** Solicitation **VDH-26-705-0387**, issued September 4, 2026, closes **October 6, 2026, 5:00 p.m. ET — 26 days. About \$11.6 million** available; awards **\$50,000 to \$1,000,000**. Avocado is not an eligible applicant; the route is as partner to a lead applicant, with real lead time to build one. See the card.

NEWLY IDENTIFIED SINCE OUR LAST EDITION

- **New York published its Rural Community Health Integration awardee cohort.** \$76.2 million; NYSDOH lists lead applicants, counties and award amounts. The question is no longer who the recipients are, but which fit Avocado best.
- **Missouri published how its ToRCH hubs will buy.** The Hub Manual states the regional coordinating network "will then begin contracting with partners/vendors on behalf of the Hubs," with a **Fall 2026 open application period for local delivery partners**. The clearest published route to a rural buyer in the forward window.
- **Comagine Health is a confirmed Idaho awardee.** Idaho's funding page lists "CLOSED 7/10/26: Maternal and Child Health Initiatives — Awardee: Comagine Health" (Perinatal Quality Collaborative and OB Readiness components; anticipated start August 14, 2026). A partner target with a named published executive.
- **Virginia Live Well, Together — Consumer Technology** enters as a priority card. The only instrument that writes the technology partner into the eligible-applicant structure. No RFA yet.

- **Idaho extended Healthcare Infrastructure Support to September 18, 2026, 11:59 p.m. MDT**, marked "Closing Deadline Extended" on the state page (previously September 13). The **\$21,954,653 Technology line is confirmed** in Idaho's August 28 release, which describes the scope as including technology solutions and software modernization.
- **Virginia Provider Productivity scope is now read against the RFA itself.** The three funded categories are provider-facing; patient-facing tools are neither listed as eligible nor excluded; the RFA's compliance clause contemplates patient-facing applications inside an approved scope. See the card.
- **Three Virginia contacts gained detail.** Brandon Rivenbark has a direct line, (804) 835-2618, and the RFA titles him Rural Hospital Program Manager (the Navigator says Flex/SHIP Program Manager). Joel Reagan (DMAS) appears on the Mobile & Hybrid Care RFA; Marcus Divers (DMAS) on Provider Productivity.

LIMITS ON COVERAGE

- Idaho's document portal serves the two prevention solicitations as unreadable files; neither deadline nor scope is verified for your top-priority Idaho item.
- No published qualifying deadline falls after October 15. Section 7 gives the structural reason.

DATED AND VERIFIED

At-a-glance deadline calendar

3

Sorted by next verified deadline, earliest first; undated open opportunities last. Announced-but-unpublished opportunities are in Section 5. Day counts are calendar days from September 10, 2026.

Deadline / type / timezone	Days remaining	State	Opportunity / status	Fit / 10	Time sensitivity	Contact	Card / official source
Sept 14, 2026, 5:00 p.m. ET — application deadline	4	VA	CareIQ Provider Productivity — Open	7	Short runway	Ben Barber, ben@vhcf.org	→ Card, Section 4 · vhcf.org/rural-health
Sept 14, 2026, 12:00 p.m. CT — response deadline	4	MN	Rural Telehealth Services RFP (SWIFT 2000018606) — Open	5	Short runway	rural.transformation.mdh@state.mn.us	MDH grants page
Sept 15, 2026, 11:59 p.m. ET — application deadline	5	VA	CareIQ Remote Patient Monitoring RFA — Open	7	Short runway	Vicki Humphreys, vhumphreys@vhha.com	→ Card, Section 4 · VHHA Foundation
Sept 16, 2026, 2:00 p.m. ET — pre-application conference (not a deadline)	6	VA	Innovative Maternal Care (VDH-26-705-0387) — Open	9	Attend; deadline is Oct 6	Brandon Rivenbark, (804) 835-2618	→ Card, Section 4
Sept 18, 2026, 5:00 p.m. ET — application deadline	8	VA	Connected to Care — Mobile & Hybrid Care (NSVRC) — Open	7	Limited runway	Brandon Davis, bdavis@nsvregion.org	→ Card, Section 4 · NSVRC RFA (PDF)
Sept 18, 2026, 11:59 p.m. MDT — application deadline (extended)	8	ID	Healthcare Infrastructure Support — Open	6	Limited runway	IdahoRHTP@dhw.idaho.gov	→ Card, Section 4 · Idaho DHW

Deadline / type / timezone	Days remaining	State	Opportunity / status	Fit /10	Time sensitivity	Contact	Card / official source
Sept 18, 2026, 5:00 p.m. (timezone not published) — application deadline	8	IL	ICCB Rural Health ROOTS (Initiative 3B) — Open, non-competitive	5	Limited runway	Ann Storey, ann.l.storey@illinois.gov	ICCB grant opportunities
Sept 25, 2026, 5:00 p.m. ET — questions due (not an application deadline)	15	VA	Innovative Maternal Care (VDH-26-705-0387) — Open	9	Question window closes	Brandon Rivenbark, (804) 835-2618	→ Card, Section 4
Oct 6, 2026, 5:00 p.m. ET — application deadline	26	VA	Connected to Care — Innovative Maternal Care (VDH-26-705-0387) — Open	9	Advance planning	Brandon Rivenbark, brandon.rivenbark@vdh.virginia.gov , (804) 835-2618	→ Card, Section 4 · VA Ways to Apply
Oct 15, 2026, 5:00 p.m. ET — initial application deadline, rolling thereafter	35	NH	Rural Primary Care Transformation (GO-NORTH) — Open	6	Advance planning	Travis Boucher, (603) 415-4258	→ Card, Section 4 · Foundation for Healthy Communities
Deadline not published by the issuing agency	Not applicable	ID	Behavioral Health Prevention — Open	6	Unknown	IdahoRHTP@dhw.idaho.gov	→ Worth Qualifying, Section 4 · Idaho DHW
Deadline not published by the issuing agency	Not applicable	ID	SUD Prevention — Open	6	Unknown	IdahoRHTP@dhw.idaho.gov	→ Worth Qualifying, Section 4 · Idaho DHW

- Innovative Maternal Care appears three times — pre-application conference, question deadline, application deadline. Only October 6 is a submission deadline.
- This calendar covers opportunities that clear your rubric. It is not a census of RHTP procurements.
- **No qualifying opportunity with a published deadline between October 16 and December 31, 2026 was identified in the reviewed sources.** A snapshot of the reviewed source set, not a claim that none exists. See Section 7.

STRONGEST OPPORTUNITY MATCHES

Priority opportunity cards

4

Ordered by fit, then urgency.

#1

Virginia Innovative Maternal Care

VIRGINIA

ADVANCE PLANNING (26 DAYS)

OPEN SOLICITATION

Fit 9/10

BEST ROUTE

Register for the **pre-application conference, September 16, 2026, 2:00 p.m. ET, Webex — 6 days**. Call **Brandon Rivenbark, Rural Hospital Program Manager, Virginia Department of Health, (804) 835-2618, brandon.rivenbark@vdh.virginia.gov**. Avocado is not an eligible applicant; ask which lead applicants want a digital health partner. Send a short capability overview covering SMS-first delivery without an app.

WHAT IT FUNDS / COVERS

About \$11.6 million; awards **\$50,000–\$1,000,000** (up to \$1M annually); at least one award per RHTP region intended. Funds rural prenatal, perinatal, postpartum and maternal behavioral health/SUD projects: care coordination and navigation; regional maternal health hubs; CHWs, doulas, peer support and home visiting; **telehealth, mobile and digital health**; transportation, language and workforce barriers. Issued by VDH under Connected to Care, Closer to Home. FORHP-rural Virginia.

DATES AND STATUS

RFA **VDH-26-705-0387**, issued **September 4, 2026**. Pre-application conference **September 16, 2:00 p.m. ET** (Webex). Questions due **September 25, 5:00 p.m. ET**; answers posted as an eVA addendum. **Applications due October 6, 2026, 5:00 p.m. ET, via eVA**. Award notices expected around **November 13, 2026**. Initial performance period through September 30, 2027; possible continuation through September 30, 2030.

LIMITS / QUALIFICATION QUESTION

Avocado is not an eligible applicant. Eligible: licensed EMS agencies, nonprofits, state, local and Tribal governments, hospitals and health systems, FQHCs, primary care clinics, RHCs, higher education — each with active SAM.gov registration and UEI. The Navigator's "entities providing maternal health technology services" describes who is served, not who may apply. Route: subcontract or named collaboration under a lead applicant, which the RFA encourages. Budget terms: **no indirect costs**, minimal direct administrative costs, no match, no pre-award costs. Awardees with a labor and delivery unit material to the project must keep it open.

Why it fits. Maternal and postpartum health; scalable decision-support layer; low-barrier delivery; rural access and equity; crisis screening and soft routing. The only instrument naming digital health, navigation, postpartum support and home visiting together, sized for a technology component, with 26 days to build a partnership. Held at 9 because Avocado cannot hold the award.

Published contact. Brandon Rivenbark, Rural Hospital Program Manager, Virginia Department of Health, brandon.rivenbark@vdh.virginia.gov, **(804) 835-2618**. Published in the RFA. The Navigator titles him Flex/SHIP Program Manager; same person, same address.

Sources. [Virginia RHTP Ways to Apply](#) — confirms Open status; no dates. RFA VDH-26-705-0387 — dates, amounts, eligibility, contact; the eVA listing disallows automated retrieval, open directly at mvendor.cgieva.com.

#2

Comagine Health

Fit 9/10

ID

ADVANCE PLANNING

AWARDED/PARTNER TARGET

BEST ROUTE

Call Comagine's Seattle office, **800-949-7536**; ask for **Dana Hawes, Chief Growth Officer**. Ask whether the Idaho OB Readiness scope includes between-visit contact with pregnant and postpartum patients, and who selects delivery partners. **No email is published for Dana Hawes.**

ROLE	Subaward recipient under Idaho's RHTP prime award; Idaho DHW administers the subgrants.
RHTP CONNECTION	Idaho DHW's funding page lists, under closed opportunities, "CLOSED 7/10/26: Maternal and Child Health Initiatives — Awardee: Comagine Health," with a Perinatal Quality Collaborative component and an OB Readiness component; anticipated award start August 14, 2026.
SCOPE AND GEOGRAPHY	National nonprofit consulting firm; offices in Portland, Seattle, Murray, Albuquerque, Anchorage, Birmingham and Washington DC. Programs in Alaska, Nevada, Oregon, Utah, Washington and Idaho , including the Idaho Perinatal Quality Collaborative.
LIMITS	The federal record for prime award RHTCMS332084 shows one subaward, \$1,198,736.00, with recipient name unreadable — not confirmed as Comagine's. Idaho names Comagine but publishes no dollar figure, and shows it winning both components against one federal subaward. Do not treat \$1,198,736 as Comagine's total. Comagine's site claims no RHTP work.

Why it fits. Maternal and postpartum health; NICU and preterm birth mitigation. Comagine improves the clinical encounter; contact between encounters is the gap you fill. Score 9: verified award, verified maternal scope, named executive, multi-state footprint.

Published contact. Dana Hawes, Chief Growth Officer (comagine.org/leadership). No email published anywhere on the site; none constructed. Office lines: Seattle 800-949-7536; Portland 503-279-0100; Anchorage 888-240-0437. **No maternal or obstetric program lead is published.**

Sources. [Idaho DHW funding opportunities](#) · [Comagine leadership](#) · [Comagine contact](#) · [Comagine programs](#) · [Idaho Perinatal Quality Collaborative](#) · [TAGGS award RHTCMS332084](#)

#3

Virginia Live Well, Together — Consumer Technology

Fit 9/10

VIRGINIA

ADVANCE PLANNING

OFFICIALLY ANNOUNCED/UPCOMING

BEST ROUTE

Email **Michael Parsons, Director of Programs, Virginia Foundation for Healthy Youth**, Michael.Parsons@healthyyouthva.org. Ask when the Consumer Technology RFA publishes and whether the digital technology company must be named at application or can be selected after award. Send a short capability overview covering SMS-first delivery without an app.

WHAT IT FUNDS / COVERS	Navigator purpose: "Procurement and technical assistance to support integration of consumer facing health apps, wearables and platforms." Administered by the Virginia Foundation for Healthy Youth. Rural Virginia. No funding amount or award count published.
DATES AND STATUS	TBD on Ways to Apply. Navigator "Fall 2026" is a planning statement, not a deadline.
LIMITS / QUALIFICATION QUESTION	No deadline, no amount. Innovative Maternal Care moved from TBD to a live RFA since our last edition; this track can too — ask now.

Why it fits. White-labeled community infrastructure for DOH agencies, FQHCs and CBOs; low-barrier delivery. Eligible applicants: "Health systems, providers, CBOs in partnership with digital technology companies." Score 9: the only instrument that writes the technology partner into the eligible-applicant structure.

Published contact. Michael Parsons, Director of Programs, Virginia Foundation for Healthy Youth, Michael.Parsons@healthyyouthva.org. No telephone published.

#4

Virginia CareIQ Provider Productivity

Fit 7/10

VIRGINIA

SHORT RUNWAY (4 DAYS)

OPEN SOLICITATION

BEST ROUTE

Email **Ben Barber, Chief Rural Health Officer, Virginia Health Care Foundation**, ben@vhcf.org. Ask whether a caregiver-facing messaging layer that absorbs routine after-hours questions is fundable under the RFA's aim of reducing inbox management, or belongs in the Consumer Technology track. Four days is too short for a new partner bid; the value is the answer and the next round.

WHAT IT FUNDS / COVERS	\$7–9 million ; no per-award cap. Purpose: "adoption of digital productivity tools, including AI-enabled decision-support, documentation, and workflow automation, to reduce administrative burden, ease provider burnout, and allow rural clinicians to focus more of their valuable time on patient care." Administered by the Virginia Health Care Foundation. FORHP-rural Virginia.
SCOPE, READ AGAINST THE RFA	The pre-application deck names three fundable categories — decision support, workflow automation, documentation — all provider-facing. Patient-facing communication is neither listed nor excluded; exclusions are generic federal cost principles. Section 6.13 contemplates "patient-facing applications" within an approved scope. So a caregiver-facing layer is fundable only if argued against the stated outcomes, not as a named category — and Virginia has a separate home for consumer-facing technology in Live Well, Together. Hence the To-Do question.
DATES AND STATUS	Applications due September 14, 2026, 5:00 p.m. ET . No extension or amendment. The companion Provider Interoperability RFA closed August 31, 2026.
LIMITS / QUALIFICATION QUESTION	Eligibility conflict: the RFA lists nonprofits, governmental entities and educational institutions; the pre-application deck omits them; the September 2 FAQ sides with the RFA. The Foundation calls this instrument Provider Productivity, not CareIQ.

Why it fits. Workforce extension; workload relief and burnout reduction; low- versus high-acuity triage. Eligible uses include reducing "time spent on clinical documentation, inbox management, prior authorization" and expanding "patient access by returning clinical time to direct care." Absorbing routine parenting questions after hours is that function. Score 7: the three named categories are provider-facing.

Published contact. Ben Barber, Chief Rural Health Officer, Virginia Health Care Foundation, ben@vhcf.org; no telephone published. Also in the RFA: Marcus Divers, Rural Health Project Manager, Virginia Department of Medical Assistance Services, Marcus.divers@dmas.virginia.gov. The Foundation's web page names no individual.

Sources. VHCF Rural Health · Provider Productivity RFA (PDF) · FAQ responses, September 2, 2026 (PDF)

#5

Virginia Mobile & Hybrid Care (NSVRC)

Fit 7/10

VIRGINIA

LIMITED RUNWAY (8 DAYS)

OPEN SOLICITATION

BEST ROUTE

Email **Brandon Davis**, Northern Shenandoah Valley Regional Commission, bdavis@nsvregion.org. The RFA names "mobile health technology organizations" as an eligible applicant type; ask whether a text-based caregiver support layer attached to a mobile unit qualifies, and whether partners can still be added.

WHAT IT FUNDS / COVERS

About **\$4,217,563**; **at least five awards**; no per-application maximum. Scope: equipment, minor renovations, technical assistance, training, personnel. Administered by NSVRC under Connected to Care, Closer to Home.

DATES AND STATUS

Applications due **September 18, 2026, 5:00 p.m. ET**. Issued August 27; awards September 30; project start October 1; subaward period ends September 30, 2027.

LIMITS / QUALIFICATION QUESTION

The RFA promised Q&A responses by September 8; **none is posted**. Its timeline also shows questions due the same day as applications — likely a drafting error. Virginia's Navigator is stale here (planning district commissions generally, Heidi Hertz as contact, "Fall 2026") while the live RFA closes in eight days.

Why it fits. Wraparound care between visits; FQHC and community health center integration. Eligible applicants include nonprofits, governmental entities, health systems, FQHCs, RHCs, educational institutions, providers, CBOs, **mobile health technology organizations**, and "other entities capable of carrying out the purposes of this RFA." Score 7: eligibility reaches a technology organization directly; funded scope is equipment-weighted.

Published contact. Brandon Davis, bdavis@nsvregion.org, NSVRC. Also on the RFA: Isabelle Drayer, Agency Program Contact, DMAS, isabelle.drayer@dmass.virginia.gov; Joel Reagan, DMAS, joel.reagan@dmass.virginia.gov. No telephones published.

Sources. [NSVRC RFA \(PDF\)](#) · [NSVRC document library](#) · [Ways to Apply](#)

#6

Virginia CareIQ Remote Patient Monitoring

Fit 7/10

VIRGINIA

SHORT RUNWAY (5 DAYS)

OPEN SOLICITATION

BEST ROUTE

Email **Vicki Humphreys**, Virginia Hospital and Healthcare Association Foundation, vhumphreys@vhha.com. Ask which eligibility statement controls for Budget Period 1 — the RFA limits applicants to hospitals and health systems; the Foundation's web page lists a wider set. The answer decides whether any route exists this year.

WHAT IT FUNDS / COVERS	"\$14.3 million... among all awarded grantees"; 2–3 awards . Navigator purpose: "Wearables, RPM devices and clinical dashboards integrated with EHR systems to support continuous patient oversight and early intervention." Administered by the VHHA Foundation. The largest published figure on any open instrument here.
DATES AND STATUS	Applications due September 15, 2026, 11:59 p.m. ET (not 5:00 p.m.). Awards October 30; Year 1 through September 30, 2027. No amendment or extension. Q&A closed August 31.
LIMITS / QUALIFICATION QUESTION	Eligibility conflict, unresolved. RFA: "hospitals and health systems in Budget Period 1." Foundation page adds free clinics, FQHCs, RHCs, independent rural providers and federally recognized Tribes. The RFA's narrower statement is more likely controlling.

Why it fits. Preventive versus reactive care; avoidable ER and urgent care reduction. The money buys continuous contact between visits — your function. Score 7: a health system applies and Avocado sits inside its package.

Published contact. Vicki Humphreys, grant applications contact, vhumphreys@vhha.com (RFA). Heather Neil, Program Manager, hneil@vhha.com (Foundation page, program inquiries). No telephones published.

Sources. [VHHA Foundation](#) · [Remote Patient Monitoring RFA \(PDF\)](#)

#7

Idaho Healthcare Infrastructure Support

Fit 6/10

IDAHO

LIMITED RUNWAY (8 DAYS)

OPEN SOLICITATION

BEST ROUTE

No separate outreach recommended. If Avocado is already in conversation with an Idaho clinic, FQHC, hospital or public health district, the technology line is the budget line to point at. Program address: IdahoRHTP@dhw.idaho.gov.

WHAT IT FUNDS / COVERS	Per Idaho's August 28 release: Cybersecurity Remediation and Advisory Consulting \$6,050,000; Technology \$21,954,653 ; Equipment \$21,469,000; Renovation \$35,070,000; Vehicles \$10,670,000; Practice Transformation, Consultation and Training \$2,000,000. The release says "\$97 million"; the lines sum to \$97,213,653. Scope includes "technology solutions; software modernization" alongside renovations and vehicles.
DATES AND STATUS	September 18, 2026, 11:59 p.m. MDT , marked "Closing Deadline Extended" on the state funding page (previously September 13). Project period ends September 30, 2027.
LIMITS / QUALIFICATION QUESTION	Eligible: rural health clinics, FQHCs, hospitals, long-term care facilities, CBOs, community providers, public health districts, Tribal governments and organizations. Avocado is not eligible. The \$97,213,653 total is not confirmed from the packet, which Idaho's portal serves as an unreadable file.

Why it fits. Scalable decision-support layer via the technology and software modernization line. State FAQ: "Technology, interoperability, cybersecurity, AI tools, and eligible EHR upgrades may be allowable when they directly support the approved rural RHTP project." Score 6: infrastructure-weighted; technology is one of six lines; Avocado is a supplier to applicants.

Published contact. No named individual. IdahoRHTP@dhw.idaho.gov is the single address for every Idaho RHTP opportunity. No telephone published.

Sources. [Idaho DHW funding opportunities](#) · [Idaho DHW news release, August 28, 2026](#) · [Healthcare Infrastructure Support FAQ \(PDF\)](#)

New Hampshire Rural Primary Care Transformation (GO-NORTH)

Fit 6/10

NEW HAMPSHIRE

ADVANCE PLANNING (35 DAYS)

OPEN SOLICITATION

BEST ROUTE

Call **Travis Boucher, Vice President, Rural Health Transformation, Foundation for Healthy Communities, (603) 415-4258**. Ask which applicants are thin on technology adoption and whether the Foundation will introduce applicants seeking technology partners. Applications roll after October 15.

WHAT IT FUNDS / COVERS	About \$15.5 million expected total awards (in the RFA, not the program page). No per-award minimum or maximum. Administered by the Foundation for Healthy Communities under GO-NORTH.
DATES AND STATUS	"Initial Application Deadline: October 15th, 2026, at 5:00 PM ET," rolling thereafter while funds remain; decisions in 45–60 days.
LIMITS / QUALIFICATION QUESTION	Eligible: FQHCs and Look-Alikes, RHCs, rural primary care practices, hospitals with primary care or rural health clinics, and organizations partnering with primary care practices. Avocado's route is as partner.

Why it fits. FQHC integration; workload relief; scalable decision-support layer. All four focus areas must be addressed; one is **Technology Adoption with Return on Investment**. Score 6, consistent with your read that this takes more work than it returns; retained for the long runway and a live telephone contact.

Published contact. Travis Boucher, direct line **(603) 415-4258**. **Email not published in the clear**; none constructed. Program address GO-NORTH-FHC@healthynh.org appears in the RFA only.

Sources. [Foundation for Healthy Communities RHTP · Rural Primary Care Transformation RFA \(PDF, final 9/2/2026\)](#)

Worth Qualifying

Two Idaho prevention solicitations, each with a specific, answerable question.

Idaho Behavioral Health Prevention | Fit: 6/10 | Timing: Deadline not published by the issuing agency | Status: Open solicitation. Posted August 21, 2026; vendor conference September 3. No application deadline published. The document folder holds a solicitation file, webinar materials, evaluation criteria, templates and an FAQ, but the solicitation text is unreadable, so deadline, funding, eligibility and scope are unverified. Program-level context from Idaho's CMS application: "Partner with rural healthcare facilities, community organizations, including faith-based entities and houses of worship, to implement evidence-based mental health and substance use prevention and education programming." **Qualification question:** does the scope reach families, parents or children, and what is the deadline? **Action:** request the packet and due date from IdahoRHTP@dhw.idaho.gov. Score 6 pending; you have flagged this as a top priority and it will be re-scored once the packet is read.

Idaho SUD Prevention | Fit: 6/10 | Timing: Deadline not published by the issuing agency | Status: Open solicitation. Posted August 21, 2026; vendor conference September 3; no due date. Idaho lists it as open and now links webinar

LEAD TIME

Emerging pipeline

5

This is where the partner lead time is. Each row is a publicly supported planned initiative, a forthcoming solicitation, or an open opportunity with no published date. Ordered by fit, then timing, then state. Forecast timing is labelled as such and is never a deadline.

State	Opportunity	Fit /10	Verified stage	Published timing / confidence	Contact	Specific next step	Official source
VA	Live Well, Together — Consumer Technology	9	Officially announced; status TBD on the state's table	"Fall 2026" — Navigator forecast, not a deadline	Michael Parsons, Michael.Parsons@healthyouthva.org	Ask when the RFA publishes and whether the technology partner must be named at application	Ways to Apply
MO	ToRCH Care Hub local delivery partner round, via the regional coordinating network "Healthier Communities Together"	8	Procedure published; 27 hub anchors named	"Fall 2026 open application period for local delivery partners" — in the state's Hub Manual; no dates set	DSS.RHTP@dss.mo.gov	Ask when the partner application period opens and how vendors are selected	ToRCH Hub Manual (PDF)
MO	Virtual Care Expansion follow-on procurement, maternal RPM track	8	RFI responses closed September 9, 2026; no follow-on published	Not published — the RFI is "for planning purposes only and will not result directly in an award"	Steven.Hoffman@dss.mo.gov	Ask whether a procurement will follow the RFI and when	MO DSS rural health
NY	Rural Community Health Integration, Budget Period 2–3 implementation funding	7	Anticipated in the state's funding guidance	"Budget Periods 2–5" — forecast; no month or date published	rchi@health.ny.gov	Use the published awardee list to identify high-fit recipients; ask the program office only about timing and structure of future implementation funding	NYSDOH RCHI Awardees List · RCHI guidance (PDF)
IL	ICAHN Technology Transformation hospital sub-grants, about \$31 million	6	Forecast; gated by a separate assessment procurement	"Q1 2027" — forecast, not a published date	Tracy Warner, Executive Director, 815-719-6206 (no email published)	Ask whether the sub-grant design will name eligible technology categories	ICAHN staff
NH	Critical Access Hospital and Acute Care Hospital RFA	5	"Coming Soon" on the administrator's page	"RFA Expected Published late August"; "Application Portal Open — Mid September" — forecast, already slipping	Travis Boucher, (603) 415-4258	Ask on the GO-NORTH call when this RFA publishes	Foundation for Healthy Communities

PARTNER ROUTES

Award winners and partner targets

6

An award is not an open solicitation. None of these organizations has published a partner deadline, and subcontracting is not established at any of them; qualifying that is the purpose of each recommended action.

Comagine Health, the top partner target, appears in Section 4.

#1

Oklahoma Health Care Authority — maternal remote patient monitoring

Fit 8/10

OK

ADVANCE PLANNING

AWARDED/PARTNER TARGET

BEST ROUTE

No named program contact is published. Write to OklahomaRHTP@health.ok.gov; ask who at the Health Care Authority owns the maternal remote patient monitoring demonstration, then ask that person what happens to a reading once a cuff transmits it.

ROLE	Administering agency under Oklahoma's prime award RHTCMS332048; reported first-tier subaward recipient.
RHTP CONNECTION AND AMOUNT	\$769,000 obligated , subaward 3409028332, executed April 3, 2026.
SCOPE	"Improve maternal health outcomes in rural OK by implementing a remote patient monitoring demonstration using connected blood pressure cuffs for up to 900 high-risk obstetrical patients/year."
LIMITS	Amount conflict, unresolved: state planned line \$800,000 vs \$769,000 obligated. Oklahoma's published recipient list covers microgrants only; attribution rests on the federal subaward report.

Why it fits. Maternal and postpartum health; NICU and preterm birth mitigation. The state has committed to continuous contact with high-risk pregnant patients, named a device and a population size. A cuff produces a reading, not the follow-up conversation that turns it into action.

Published contact. No named individual. OklahomaRHTP@health.ok.gov is the program address; no telephone. Tracey Douglas, Director of Grants Management Financial Services, tracey.douglas@health.ok.gov, 405.590.2108 — grants administration, not the program lead.

Sources. USAspending subawards, RHTCMS332048 · Oklahoma RHTP Initiative Funding Summary (PDF) · Oklahoma RHTP

#2

Oklahoma State University — OSU Medicine Virtual Care OB/GYN

Fit 8/10

OK

ADVANCE PLANNING

AWARDED/PARTNER TARGET

BEST ROUTE

Call OSU Medicine partnership inquiries, **918-200-6960**; ask for **Jared M. Droze, MHA, Director of Virtual Care**. Ask how the virtual OB/GYN clinic keeps contact with patients between scheduled encounters. **Emails on that page are machine-obfuscated and not reproduced.**

RHTP CONNECTION AND AMOUNT	\$2,252,567 obligated , subaward 3409028355, April 8, 2026. A second \$13,086,285 subaward (3409028375, May 13, 2026) covers community paramedicine training, maternal telemedicine support and residency expansion; maternal share unstated.
SCOPE	"osu medicine virtual care-obgyn clinic operations... to deliver comprehensive obstetric and gynecologic services through a virtual care model."
LIMITS	OSU Medicine's virtual care page lists no obstetric, gynecologic or women's health service . The funded clinic has no public page; the state's nearest line is "Maternal-Fetal Medicine (MFM) Telehealth Expansion," \$6,600,000. Naming mismatch stated, not resolved.

Why it fits. Maternal and postpartum health; wraparound care between visits. A virtual OB/GYN clinic is a scheduled-encounter model; between encounters is where a text-first layer operates.

Published contact. Jared M. Droze, MHA, Director of Virtual Care; Bradley Anderson, D.O., Medical Director, Virtual Care; Mike Shea, DHA, Executive Director of Strategic Initiatives. Telephones: 918-200-6960 (partnership inquiries); 918-584-4377 (rural health telehealth office).

Sources. [USAspending subawards, RHTCMS332048](#) · [OSU Medicine Virtual Care](#) · [OSU Rural Health Telehealth](#)

#3

New York RCHI awardee cohort

Fit 7/10

NEW YORK

ADVANCE PLANNING

AWARDED/PARTNER TARGET

BEST ROUTE

Use NYSDOH's published Rural Community Health Integration Awardees List to identify and qualify recipients with the strongest maternal, family-navigation, behavioral-health or digital-care overlap. The list gives lead applicant, counties and award amount.

ROLE	Cohort of subaward recipients under New York's RHTP award, administered by the New York State Department of Health.
RHTP CONNECTION AND AMOUNT	\$76.2 million across 90 awards to 56 organizations , announced September 4, 2026 — about \$22.7 million planning, \$53.5 million implementation — from New York's \$212 million first-year award.
SCOPE	"Establish coordinated rural health partnerships that facilitate/finance communities to develop comprehensive care coordination enhancing patient access to a wide range of providers across the care continuum, effectively addressing health and social needs."
LIMITS	The list does not establish that any recipient seeks a technology partner or has an open subcontracting process. Qualification targets, not confirmed buyers.

Why it fits. Wraparound care between visits; social determinants navigation. Fifty-six rural organizations just funded to build care coordination, most of it implementation money. Score 7: the list gives no project descriptions, so fit must be qualified organization by organization.

Published contact. rchi@health.ny.gov and RHTP@health.ny.gov. Karen Madden, Center for Health Care Policy and Resource Development, is named in the funding guidance; no telephone published.

Sources. [NYSDOH RCHI Awardees List](#) — page marked Revised: September 2026 · [Governor's announcement, September 4, 2026](#) · [NYSDOH Transforming Rural Healthcare](#) · [RCHI funding guidance \(PDF\)](#)

#4

Missouri ToRCH Care Hub anchors

Fit 7/10

MO

ADVANCE PLANNING

AWARDED/PARTNER TARGET

BEST ROUTE

Email torch@brhc.org at Bothwell Regional Health Center (Hub 13), which publishes a live ToRCH application route for community organizations and names **Elizabeth Green, ToRCH Program Manager**. Ask what technology and caregiver-support services the hub expects to procure through the network.

ROLE

Hub anchors, **not award recipients**; participation "is subject to execution of a Hub Anchor Participation Agreement." Contracting runs through the regional coordinating network.

RHTP CONNECTION

27 hub anchors named July 17, 2026, from 41 applications; 19 are hospitals. Hub Manual: the network "will then begin contracting with partners/vendors on behalf of the Hubs"; hubs "open an application period to select specific implementation partners," who contract with "Healthier Communities Together."

LIMITS

The anchor list publishes no contacts; the Hub Manual only placeholders. No hub-by-hub procurement calendar. Not to be confused with Missouri's 2023 six-hospital ToRCH pilot.

Why it fits. White-labeled community infrastructure; wraparound care between visits. Twenty-seven named anchors, a published procurement route and a Fall 2026 partner window — the clearest forward path to a rural buyer on this file.

Published contact. torch@brhc.org, with Elizabeth Green named ToRCH Program Manager and main line 660.826.8833, on Bothwell's ToRCH page. DSS.RHTP@dss.mo.gov is the state program address.

Sources. Missouri RHTP Hub Anchors, July 17, 2026 (PDF) · ToRCH Hub Onboarding and Operations Procedures Manual (PDF) · Bothwell ToRCH community organizations

#5

AtlantiCare Regional Medical Center

Fit 7/10

NJ

ADVANCE PLANNING

AWARDED/PARTNER TARGET

BEST ROUTE

No named individual is published. Call the childbirth center line, **(888) 569-1000**; ask who leads the doula integration model funded under New Jersey's rural health transformation allocation.

ROLE

Allocation recipient under New Jersey's RHTP award, administered by the New Jersey Department of Human Services.

RHTP CONNECTION AND AMOUNT

\$790,478 allocated, July 31, 2026: "Establish a doula integration model to increase the delivery of patient-centered prenatal and perinatal care through standardized practices, shared decision making, culturally responsive care, and individualized birth plans." AtlantiCare also holds \$2,045,763 for mobile integrated health and \$1,460,000 at each of two campuses for digital infrastructure.

Why it fits. Maternal and postpartum health; parent self-efficacy. A system already spending RHTP money on both maternal care and digital capability.

Published contact. mahs.njrht@dhs.nj.gov is New Jersey's only published RHT contact; no program lead named. AtlantiCare's childbirth center publishes (888) 569-1000 and a Crib Support Line, 609-404-3835; no staff named.

Sources. [New Jersey Rural Health Transformation · NJ RHT funding allocations, July 31, 2026 \(PDF\)](#) · [AtlantiCare Roger B. Hansen Center for Childbirth](#)

#6

Foundation for a Healthy Oklahoma — community health worker expansion

Fit 6/10

OK

ADVANCE PLANNING

AWARDED/PARTNER TARGET

BEST ROUTE

Call the Oklahoma Hospital Association, **(405) 427-9537**; ask for **Hayley Warren, Manager of Rural Health**. Ask what tools the 30 new community health workers will use to stay in contact with families between visits. **Directory emails are encrypted links and not reproduced.**

ROLE	Subaward recipient under Oklahoma's prime award; hosted and staffed by the Oklahoma Hospital Association.
RHTP CONNECTION AND AMOUNT	\$4,586,721.33 obligated , subaward 3409028256, executed March 4, 2026. Purpose: expand "use of community health workers (CHWS) in rural communities to further demonstrate effectiveness and expand coverage of chws across payer types."
LIMITS	Amount conflict, unresolved: state planned line \$4,300,000 for "30 additional CHWs" vs \$4,586,721.33 obligated. No CHW or RHTP page on the Foundation's site section.

Why it fits. Workforce extension; workload relief. A CHW in a rural hospital is the human end of the same between-visit contact model.

Published contact. Eric Finley, Executive Director of the Foundation; Hayley Warren, Manager of Rural Health; both on the Association's staff directory with office line (405) 427-9537.

Sources. [USAspending subawards, RHTCMS332048](#) · [Oklahoma Hospital Association staff directory](#) · [Oklahoma RHTP Initiative Funding Summary \(PDF\)](#)

CONTEXT

Related verified intelligence

Ordered by relevance to the highest-ranked opportunities, then by state.

Virginia's status table is a status signal, not a calendar. It shows Innovative Maternal Care as Open with its solicitation number but no deadline, amount or dates for any track. Everything datable lives in the RFA behind the eVA link. Affects the → Innovative Maternal Care card, Section 4.

A Navigator target audience is not an eligible-applicant list. The Navigator names "entities providing maternal health technology services"; the RFA's eligible applicants are EMS agencies, nonprofits, governments, hospitals and health systems, FQHCs, primary care clinics, RHCs and higher education. Only the RFA governs who may apply. Carry this into every Virginia track. Affects both Virginia priority cards.

Virginia's Navigator is stale where a live RFA exists. On Mobile & Hybrid Care it names the planning district commissions generally, gives Heidi Hertz as contact and "Fall 2026" timing, while NSVRC's RFA closes September 18. Treat the Navigator as roadmap and the administrator's document as authority. Affects the → Mobile & Hybrid Care card, Section 4.

Idaho corrected a deadline on its page, not in the packet. The September 13 to 18 extension appears only as a parenthetical on the funding page; the original date only in an August 28 release. Idaho's portal still serves solicitation packets as unreadable files, which is why two Idaho opportunities carry no verified deadline. Affects the → Healthcare Infrastructure Support card, Section 4 and both → Worth Qualifying, Section 4 entries.

Missouri's Virtual Care Expansion instrument was an RFI, not a funding opportunity — "for planning purposes only and will not result directly in an award." One of its two tracks was maternal RPM (hypertensive disorders, gestational diabetes, high-risk pregnancy). Responses were due September 9; no follow-on solicitation, award or timeline is published. It sits in the pipeline, not the calendar.

Missouri's Children's Trust Fund home-visiting round closed with no awardees published. \$588,000 annually — up to \$100,000 for model fees and training, up to \$488,000 for expansion of nurse-based, HomVEE-approved home visiting. Closed August 23, 2026. No award announcement. The Fund is the administrator, not an awardee.

The federal Year 1 timetable explains the compressed calendar but does not set a uniform cycle. Budget Period 1 ends **October 30, 2026**; state materials say Year 1 funds must be obligated by then; CMS allows approved Budget Period 1 costs to be paid through September 30, 2027. Solicitation timing still varies by state; Budget Period 2 timing is not established. Keep allocation (plan), obligation (commitment), award (named recipient) and payment distinct.

Several administrator and partner sites obscure email addresses. New Hampshire and some Oklahoma organization pages use email-protection or encrypted links. Where an address is not published in the clear, this report says so and does not reconstruct one. Affects the → GO-NORTH card, Section 4 and the Oklahoma partner cards.

CONTACT SUMMARY

Contact summary

8

Consolidated and deduplicated. Published sources only.

CIVIC OPERATOR

Avocado Verified Intelligence · September 10, 2026 · Confidential 17/20

Opportunity / partner	Contact	Title / organization	Email	Phone	Notes	Public source
VA Innovative Maternal Care (VDH-26-705-0387)	Brandon Rivenbark	Rural Hospital Program Manager, Virginia Department of Health	brandon.rivenbark@vdh.virginia.gov	(804) 835-2618	Published in the RFA. The Navigator gives Flex/SHIP Program Manager for the same person and address	RFA VDH-26-705-0387; Ways to Apply
VA Live Well, Together — Consumer Technology	Michael Parsons	Director of Programs, Virginia Foundation for Healthy Youth	Michael.Parsons@healthyouthva.org	Not published	Published	RFA Navigator V8 (PDF)
VA CareIQ Provider Productivity	Ben Barber	Chief Rural Health Officer, Virginia Health Care Foundation	ben@vhcf.org	Not published	Published in the RFA; the Foundation's web page names no individual	Provider Productivity RFA (PDF)
VA CareIQ Provider Productivity	Marcus Divers	Rural Health Project Manager, Virginia Department of Medical Assistance Services	Marcus.divers@dmass.virginia.gov	Not published	Published in the RFA. New since our last edition	Provider Productivity RFA (PDF)
VA CareIQ Remote Patient Monitoring	Vicki Humphreys	Contact for grant applications, VHHA Foundation	vhumphreys@vhha.com	Not published	Published in the RFA	RPM RFA (PDF)
VA CareIQ Remote Patient Monitoring	Heather Neil	Program Manager, VHHA Foundation	hneil@vhha.com	Not published	Published	VHHA Foundation
VA Mobile & Hybrid Care	Brandon Davis	Northern Shenandoah Valley Regional Commission	bdavis@nsvregion.org	Not published	Published in the RFA	NSVRC RFA (PDF)
VA Mobile & Hybrid Care	Isabelle Drayer	Agency Program Contact, Virginia Department of Medical Assistance Services	isabelle.drayer@dmass.virginia.gov	Not published	Published in the RFA	NSVRC RFA (PDF)
VA Mobile & Hybrid Care	Joel Reagan	Virginia Department of Medical Assistance Services	joel.reagan@dmass.virginia.gov	Not published	Published in the RFA. New since our last edition	NSVRC RFA (PDF)
VA RHTP program, all Virginia tracks	Heidi Hertz	Director, Rural Health Transformation, Office of the Governor of Virginia	Heidi.Hertz@governor.virginia.gov	Not published	Published. Program-level, not track-specific	RFA Navigator V8 (PDF)
ID Healthcare Infrastructure Support; ID Behavioral Health Prevention; ID SUD Prevention	No named individual published	Idaho Department of Health and Welfare, RHTP	IdahoRHTP@dhw.idaho.gov	Not published	Published in the clear with the correct .gov domain. Idaho names no program lead	Idaho DHW funding opportunities
NH Rural Primary Care Transformation (GO-NORTH)	Travis Boucher	Vice President, Rural Health Transformation, Foundation for Healthy Communities	Not reproduced — hidden behind email protection	(603) 415-4258 (direct)	Name, title and telephone published in the clear	Foundation for Healthy Communities
NH Rural Primary Care Transformation (GO-NORTH)	No named individual — program address	Foundation for Healthy Communities (GO-NORTH)	GO-NORTH-FHC@healthynh.org	Not published	Published in the RFA, not on the program page	GO-NORTH RFA (PDF)
MN Rural Telehealth Services RFP	No named individual on the grants page	Minnesota Department of Health, Office of Rural Health and Primary Care	rural.transformation.mdh@state.mn.us	651-201-3838 / 800-366-5424 (general office line)	Published	MDH grants page
MN Rural Telehealth Services RFP	Zomi Bloom	State Program Administrator Principal, Minnesota Department of Health	grants.ruraltransformation.mdh@state.mn.us	Not published	Published in the RFP PDF. Address conflicts with the grants page; stated, not resolved	Rural Telehealth Services RFP (PDF)
IL ICCB ROOTS (Initiative 3B) CIVIC OPERATOR	Ann Storey	Director for Healthcare Programs, Illinois Community College Board	ann.i.storey@illinois.gov	Not published	Published on the ICCB page and in September 10, 2026 both NOFOs	ICCB grant opportunities

Opportunity / partner	Contact	Title / organization	Email	Phone	Notes	Public source
Comagine Health (partner target)	Dana Hawes	Chief Growth Officer, Comagine Health	Not published anywhere on the organization's site	800-949-7536 (Seattle office, not a direct dial)	Name and title published; no address constructed	Comagine leadership · Comagine contact
OK maternal remote patient monitoring	No named individual — program address	Oklahoma State Department of Health, RHTP	OklahomaRHTP@health.ok.gov	Not published	Published	Oklahoma RHTP
OK RHTP award administration	Tracey Douglas	Director of Grants Management Financial Services, Oklahoma State Department of Health	tracey.douglas@health.ok.gov	405.590.2108	Published in the state's RHTP project narrative. Grants administration, not the program lead	Oklahoma RHTP Project Narrative (PDF)
OK OSU Medicine Virtual Care OB/GYN	Jared M. Droze, MHA	Director of Virtual Care, OSU Medicine	Not reproduced — machine-obfuscated on the page	918-200-6960 (partnership inquiries)	Name, title and telephone published	OSU Medicine Virtual Care
OK community health worker expansion	Hayley Warren	Manager of Rural Health, Oklahoma Hospital Association	Not reproduced — directory addresses are encrypted links	(405) 427-9537	Name, title and office line published	OHA staff directory
NJ AtlantiCare doula integration model	No named individual — program address	New Jersey Department of Human Services, NJ RHT	mahs.njrht@dhs.nj.gov	Not published	Published. New Jersey names no program lead	New Jersey Rural Health Transformation
NY RCHI awardee cohort	No named individual on the program page	New York State Department of Health, RCHI	rchi@health.ny.gov ; RHTP@health.ny.gov	Not published	Published. Karen Madden is named in the funding guidance with no contact details	NYSDOH Transforming Rural Healthcare
MO ToRCH Care Hub anchors	Elizabeth Green	ToRCH Program Manager, Bothwell Regional Health Center (Hub 13)	torch@brhc.org (program address published alongside her name)	660.826.8833 (main line)	Published on the hospital's ToRCH page	Bothwell ToRCH community organizations
MO ToRCH program; MO Virtual Care Expansion follow-on	Steven Hoffman	Missouri Department of Social Services — named point of contact, RFI 0000000012SL; no title published	Steven.Hoffman@dss.mo.gov ; DSS.RHTP@dss.mo.gov	Not published	Published	MO DSS rural health

Twenty-five rows, nineteen named individuals. Where no named individual is published, the table says so. Where a site obscures addresses — Travis Boucher, Dana Hawes, Jared Droze, Hayley Warren — none is reproduced or constructed from a naming pattern. Two general program inboxes are labelled as such. An organizational affiliation does not establish a program-specific role. Confirm incumbents before use; note the Rivenbark title discrepancy.

Sources, snapshot note, and feedback

VIRGINIA

- [Virginia Rural Health Transformation, Ways to Apply and RFA status table](#) — confirms Innovative Maternal Care as Open with solicitation VDH-26-705-0387; publishes no dates
- RFA **VDH-26-705-0387**, Innovative Maternal Care, issued September 4, 2026 — source for the October 6 deadline, conference and question dates, \$11.6M availability, \$50,000–\$1,000,000 award range, eligible-applicant list and contact. The eVA listing disallows automated retrieval: mvendor.cgieva.com
- [Virginia RHT RFA Navigator V8 \(PDF\)](#)
- [VHHA Foundation, Virginia Rural Health Transformation](#)
- [VHHA Remote Patient Monitoring RFA \(PDF\)](#) — key dates table; eligibility at "Eligible entity types"
- [Virginia Health Care Foundation, Rural Health](#)
- [VHCF Provider Productivity RFA \(PDF\)](#) — purpose §1.2; eligible uses §2.1; eligible applicants §1.4; HIT and interoperability §6.13
- [VHCF Provider Interoperability and Provider Productivity FAQ responses, September 2, 2026 \(PDF\)](#)
- [NSVRC Mobile & Hybrid Care RFA \(PDF\)](#) · [NSVRC document library](#)

IDAHO

- [Idaho DHW, Funding Opportunities for Rural Health Transformation](#)
- [Idaho DHW news release, Healthcare Infrastructure Support, August 28, 2026](#) — activity-line breakdown and the original September 13 date
- [Idaho DHW Healthcare Infrastructure Support FAQ \(PDF\)](#)
- [Idaho RHTP application to CMS \(PDF\)](#) — program-level context only
- [TAGGS award RHTCMS332084](#)

COMAGINE HEALTH

- [Leadership](#) · [Contact and offices](#) · [Programs](#) · [Idaho Perinatal Quality Collaborative](#)

MISSOURI

- [MO DSS / MO HealthNet rural health](#)
- [Missouri RHTP Hub Anchors, July 17, 2026 \(PDF\)](#)
- [ToRCH Care Hub Onboarding and Operations Procedures Manual, June 30, 2026 \(PDF\)](#) — contracting route §6 and §6.1; Stage Gates 2 and 3
- [Bothwell Regional Health Center, ToRCH community organizations](#)
- [Missouri Children's Trust Fund, rural health transformation home visiting announcement, June 26, 2026](#) · [CTF funding opportunities](#)

NEW YORK

- [Governor Hochul announces \\$76 million, September 4, 2026](#)
- [NYSDOH, Transforming Rural Healthcare · RCHI funding guidance \(PDF\)](#)
- [NYSDOH Rural Community Health Integration Awardees List](#) — lead applicants, counties and award amounts; marked Revised: September 2026

OKLAHOMA, NEW JERSEY, ILLINOIS, MINNESOTA, NEW HAMPSHIRE

- [USAspending first-tier subaward report, Oklahoma prime award RHTCMS332048](#)
- [Oklahoma RHTP](#) · [Oklahoma RHTP Funding](#) · [Initiative Funding Summary \(PDF\)](#) · [Project Narrative \(PDF\)](#)
- [OSU Medicine Virtual Care](#) · [OSU Rural Health Telehealth](#) · [Oklahoma Hospital Association staff directory](#)
- [New Jersey Rural Health Transformation](#) · [NJ RHT funding allocations \(PDF\)](#) · [AtlantiCare Center for Childbirth](#)
- [Illinois Community College Board grant opportunities](#) · [RHT ROOTS \(3B\) NOFO \(PDF\)](#)
- [Minnesota MDH rural health transformation grants and RFPs](#) · [Rural Telehealth Services RFP \(PDF\)](#)
- [Foundation for Healthy Communities, New Hampshire RHTP](#) · [Rural Primary Care Transformation RFA \(PDF\)](#)
- [ICAHN staff directory](#)

ONE DOCUMENT.

The → [Deadline Calendar, Section 3](#) and → [Emerging Pipeline, Section 5](#) are embedded here; there is no separate attachment.

SNAPSHOT NOTE.

Entries are snapshots as of the edition date, not continuously tracked. Material Q&A updates are reflected in subsequent editions once verified; consult the linked official source for current details. Virginia will post answers to Innovative Maternal Care questions as an eVA addendum after September 25.

RESEARCH CAVEATS.

Figures are as published at the edition date unless a row says otherwise. Virginia's Navigator and status table carry no version stamp, publish no dates, and disagree with the live RFAs on two tracks. Idaho's portal serves its solicitation packets as unreadable files, so two Idaho opportunities carry no verified deadline or quotable scope. The federal subaward report covers first-tier subawards of \$30,000 and up only and is self-reported by the state; smaller awards and second-tier subcontracts never appear in it. States and administrators move deadlines without notice.